

Universitätsklinikum Bonn — Department of Internal Medicine

Discharge Summary

Patient: Test Patient | DOB: 1990-01-01 | MRN: TP-00423

Admission: 2026-06-20 | Discharge: 2026-06-24 | Attending: Dr. Test

Reason for Admission

Fatigue, shortness of breath on exertion, and pallor for 2 weeks. Outpatient labs showed hemoglobin of 9.8 g/dL prompting admission for further workup.

Hospital Course

Patient underwent upper and lower endoscopy to rule out occult GI bleeding (negative). Iron studies confirmed iron-deficiency anemia (ferritin 8 ng/mL, low serum iron, elevated TIBC). Started on IV iron sucrose with good tolerance; hemoglobin rose to 10.6 g/dL by discharge. Cardiology consulted for exertional dyspnea — echocardiogram normal, attributed to anemia.

Discharge Medications

Ferrous sulfate 325 mg PO once daily, Vitamin C 500 mg PO once daily (improves iron absorption), continue home Metformin 500 mg BID and Atorvastatin 20 mg nightly.

Follow-Up

Outpatient hematology follow-up in 4 weeks with repeat CBC and iron panel. Primary care follow-up in 2 weeks. Return to ER for chest pain, severe dyspnea, or melena.

Dictated by: Dr. Test, MD Cosigned: Dr. Schmidt, Attending Physician